

TITLE: Request for Information (RFI) Administration for Children and Families (ACF), Office of Refugee Resettlement (ORR), Unaccompanied Alien Children Bureau (UACB), Special Population Continuum of Care (CoC) Residential Care Network

1.0 PURPOSE

This is a RFI ONLY and is for market research purposes. This is NOT a solicitation for proposals, proposal abstracts, or quotations. The Government is seeking capability statements to obtain knowledge and information on potential vendor capability, capacity, and interest for project planning purposes. The Government will not accept unsolicited proposals in response to this notice.

The Administration for Children and Families (ACF), Office of Refugee Resettlement (ORR) is conducting market research to identify potential sources capable of providing a Continuum of Care (CoC) network solution for Unaccompanied Alien Children (UAC) who require special population levels of care.

2.0 BACKGROUND

ORR is responsible for the care and placement of UAC referred by federal agencies, most often by the Department of Homeland Security (DHS). A UAC has no lawful immigration status in the United States; has not attained 18 years of age; and has no parent or legal guardian in the United States or no parent or legal guardian in the United States who is available to provide care and physical custody (defined at 6.U.S.C 279(g)(2)).

ORR has cared for more than 560,000 children, incorporating child welfare principles and provisions established by the Flores Settlement Agreement in 1997, the Trafficking Victims Protection Act of 2000 and its reauthorization acts, the William Wilberforce Trafficking Victims Protection Reauthorization Act (TVPRA) of 2005 and 2008, the Lucas R. Settlement Agreement, and [45 CFR Part 410](#). ORR funds a network of care provider facilities to accept the physical custody of UAC. A care provider facility is any physical site that houses one or more UAC in ORR custody and is operated by an ORR-funded program that provides residential services for UAC.

UAC arrive from diverse backgrounds, and of all ages with unique and individual needs. It is not uncommon for UAC to have experienced trauma in their country of origin or while traveling to enter the United States. With this, some UAC meet certain criteria that requires placement in care that can address their unique and individual challenges, including criminal history and mental or behavioral health needs. ORR takes into consideration the unique nature of each child's situation and incorporates child welfare principles when making placement, clinical, case management, and release decisions. While determining least restrictive care settings, ORR takes into consideration factors including whether a child is a danger to themselves or other UAC, danger to others or danger to the community, and whether there is a risk the minor child may try to flee or run away from ORR custody. Residential care designed to address

these challenges include Diagnostic Center Care (DC); Psychiatric Residential Treatment Facility Care (PRTF); Residential Treatment Center Care (RTC); Therapeutic Group Home Care (TGH); Heightened Supervision Facility (HSF), also known as Staff Secure; and Partial Hospitalization Program (PHP). All levels of special population care are provided in humane, therapeutic, culturally competent environments while delivering high quality comprehensive services catered to the needs of each individual child. Decisions for placement in a special population level of care is made when a UAC arrives in the United States with a known or self-disclosed criminal history or record or presents with psychiatric or behavioral health needs that cannot be addressed in outpatient settings or less restrictive levels of care.

The Government is seeking industry information on the availability of existing state-licensed residential care providers for minors located within the contiguous United States, with capacity to provide a continuum of residential care. The need is immediate, as such, an operational ready site and care providers experienced in network management with capacity to ramp up as soon as possible upon a contract award, but not longer than 120 calendar days, is desired.

3.0 SCOPE

In cases where UAC are determined to meet criteria for placement in a special population level of care, ORR utilizes experienced qualified care provider facilities that are state-licensed and nationally accredited with proven success in the delivery of comprehensive quality programming for youth with complex behavioral and mental health care needs.

The Government's expectation is that a network strategy connects licensed and accredited care facilities planned as a community of practice to ensure availability of beds and compliance with all governing legislation and policies for serving youth with complex behavioral and mental health needs. The network will employ industry best practices and be located in a cohesive local geographic area or region to ensure timely admissions, diagnostics, continuity of care for UAC with complex needs, smooth and timely transfers between levels of care as may be clinically necessary, and safe and timely discharges and family unifications. With this, ORR is seeking innovative approaches and networking strategies for a continuum of care (CoC) network that ensures availability of timely admission to least restrictive levels of care for UAC who meet placement criteria within the defined continuum of care.

The desired network management strategy for a special population continuum of care shall provide a solution that will leverage known network management business strategies and continuity of care strategies to:

1. Offer fiscal efficiency on daily bed rates and payment structure for occupied beds with methodology for funding unoccupied or "held" beds.
2. Offer industry best practices for continuity of care for youth with complex behavioral and mental health needs.
3. Ensure timely admission and transfers to least restrictive, most integrated setting appropriate to

their needs and in their best interests.

4. Ensure compliance with all federal, state, and local licensing and governing requirements for serving youth with complex behavioral and mental health needs.
5. Ensure provision of all ORR required comprehensive services and compliance with ORR UAC Bureau policies.

<https://acf.gov/orr/policy-guidance/unaccompanied-children-bureau-policy-guide>

The Table below depicts the minimum number of beds per level of care, a continuum of care (CoC) must include, and the maximum number of beds per level of care a continuum of care must not exceed. The table also notes corresponding average lengths of stay in custody and care for each level of care.

Level of Care	Minimum Beds	Maximum Beds	Average Length of Stay
Diagnostic Center	10	50	30 Days
Psychiatric Residential Treatment Facility	10	30	72 Days
Residential Treatment Center	10	70	72 Days
Heightened Supervision Facility	35	120	72 Days
Therapeutic Group Home	8	30	72 Days
Partial Hospitalization Program	15	50	21 Days

The Government anticipates awarding one or more contracts; however, each awardee must provide at least the minimum number of beds for all levels of care within the continuum.

The continuum of care shall include the following levels of complex care:

Diagnostic Center (DC): Provides humane and secure environments for UAC who have conflicting mental health diagnosis or limited information regarding violent criminal behavior that delivers comprehensive evaluations by licensed medical practitioners to determine least restrictive placement options as alternative to secure placement in juvenile detention. DCs provide temporary and safe custody of UAC for diagnostic purposes not to be utilized for more than 30 days, or other periods of time prescribed by a licensed medical provider. The purpose is to accept and accurately assess UAC with undiagnosed or misdiagnosed mental health issues. Criteria for DC Placement Considers:

- Charged, chargeable, or convicted of a crime, or subject of delinquency proceedings, adjudicated delinquent, or chargeable with a delinquent act.
- Committed or have made credible threats to commit, a violent or malicious act while in ORR custody.
- Engaged in conduct unacceptably disruptive of normal functioning in an HSF that transfer may be necessary to ensure welfare of UAC or others.
- Self-disclosed violent criminal history prior to placement in ORR custody requiring further assessment.

- Exhibits violent behavior as a result of an underlying mental health condition that cannot be managed in a less restrictive setting.
- Committed sexual abuse, where there is coercion by overt or implied threats of violence.

DC Purpose:

- Deliver comprehensive evaluations to determine least restrictive, most integrated placement appropriate to their needs and in their best interests.
- Alternative secure placement to juvenile detention.
- Provide humane and secure environments for UAC who have conflicting mental health diagnosis or limited information regarding violent criminal behavior.
- When appropriate, makes safe release recommendations to an ORR-vetted sponsor.

Psychiatric Residential Treatment Facility (PRTF): A secure unit to provide a humane and therapeutic environment for UAC between the ages of 12 to 17 with serious psychiatric or psychological issues that result in aggressive behaviors towards others and themselves, and behaviors that cannot be managed in RTC placement. While PRTFs are generally described as serving youth ages 12–17, the criteria for placement is clinical necessity and risk level, rather than age alone.

UAC who meet criteria for PRTF Care receive care and custody, assessment and diagnosis, and treatment and discharge plans. Criteria for PRTF Placement Considers:

- Has shown no reasonable progress in alleviation of mental health symptoms after significant period in outpatient treatment; and demonstrate behavior that is a result of underlying mental health symptoms and/or diagnosis and cannot be managed in an outpatient setting.
- Requires therapeutic-based intensive supervision as result of mental health symptoms and/or diagnosis to prevent independent participation in daily schedule of activities.
- Present continued real risk of harm to self, others, or community despite short-term clinical interventions.

PRTF Purpose:

- Provide humane and therapeutic environment for UAC between the ages of 12 to 17 with serious psychiatric or psychological issues that result in aggressive behaviors towards others and themselves, and behaviors cannot be managed in RTC placement.
- When appropriate, make safe release recommendations to least restrictive care or to vetted sponsor.
- Provide care and custody; assessment and diagnosis; and develop treatment and discharge plans.

Residential Treatment Center (RTC): Care that provides a humane and therapeutic environment for UAC with serious mental health issues, while delivering comprehensive services. Residential treatment care provides temporary and safe custody of UAC. It is a sub-acute, time limited, interdisciplinary, psycho-educational, and therapeutic 24-hour-a-day structured program with community linkages, provided through non-coercive, coordinated, individualized care, specialized services, and

interventions. RTCs provide highly customized care and services to individuals following either a community-based placement or more intensive intervention, with the aim of moving individuals toward a stable, less intensive level of care or independence. RTCs are a type of secure facility and are not a standard program. While RTCs are generally described as serving youth ages 12–17, the criteria for placement is clinical necessity and risk level, rather than age alone. Criteria for RTC Placement Considers:

- Have not shown reasonable progress in alleviation of mental health symptoms after significant period of time in outpatient treatment.
- Demonstrate underlying mental health symptoms or diagnosis that cannot be managed in outpatient setting; or requires therapeutic-based intensive supervision due to mental health symptoms and/or diagnosis that prevents independent participation in daily schedule of activities.
- Behavior presents a continued and real risk of harm to self, others, or the community despite implementation of short-term clinical interventions.

RTC Purpose:

- Provide humane and therapeutic environment for UAC while delivering comprehensive services to UAC with serious mental health issues.
- For UAC who have serious psychiatric or psychological issues that cannot be addressed in an outpatient setting and are danger to self or others.
- When appropriate, make safe release recommendations to an ORR-vetted and approved sponsor.
- When appropriate, make safe transfer recommendation to less restrictive setting.

Therapeutic Group Home (TGH): Not considered a restrictive level of care but is a required therapeutic part of the CoC for continuity and efficiency of care. Criteria for TGH Placement Considers:

- Ages 12 to 17 who are not a danger to self or others.
- Would benefit from enhanced behavioral health services.
- Require a smaller care setting.
- Do not require RTC level of care.

TGH Purpose:

- Provide humane and therapeutic environment for UAC while delivering comprehensive mental health services and when appropriate, transfer to least restrictive settings.
- Make safe release recommendations to an ORR-vetted and approved sponsor.

Heightened Supervision Facility (HSF): A facility that is operated by a program, agency, or organization licensed by an appropriate State agency, or that meets the requirements of State licensing that would otherwise be applicable if it is in a State that does not allow state licensing of programs providing care and services to unaccompanied alien children, and that meets the standards for standard programs set forth in 45 CFR § 410.1302, and that is designed for an unaccompanied alien child who

requires close supervision but does not need placement in a secure facility, including a residential treatment center (RTC). It provides 24-hour supervision, custody, care, and treatment. It maintains stricter security measures than a shelter, such as intensive staff supervision, in order to provide supports, manage problem behavior, and prevent children from running away. A heightened supervision facility may have a secure perimeter but shall not be equipped internally with major restraining construction or procedures typically associated with juvenile detention centers or correctional facilities. Historically, this type of facility was referred to as “staff secure” in ORR sub-regulatory guidance and information collections. While HSF are generally described as serving youth ages 12–17, the criteria for placement is clinical necessity and risk level, rather than age alone. Criteria for HSF Placement Considers:

- UAC who are disruptive to normal functioning of shelter care facilities such that transfer is necessary to ensure safety and welfare of others.
- Have displayed pattern of severity of behavior requiring increase in supervision by trained staff.
- UAC who are an escape risk.
- Have non-violent criminal or delinquent history not warranting placement with Secure Care provider.

HSF Purpose:

- Provide humane, therapeutic environment for UAC ages 12 to 17.
- Deliver comprehensive services to UAC who have behavioral and/or criminal issues that require close supervision, but do not require placement in a secure provider placement.
- When necessary, transfer to least restrictive setting.
- When appropriate recommend safe release to an ORR-vetted and approved sponsor.

Partial Hospitalization Program (PHP): Well-established mental health program that typically meets 5 days per week for several hours a day. UAC are seen in group and individual treatments and engage in structured activities. The program is used to step down from acute hospitalization or to prevent acute admission and is designed to prevent unnecessary hospitalization and to shorten acute stays. UAC attend the program during the day and remain in shelter or least restrictive levels of care at night. UAC typically attend between 3 and 4 weeks. Criteria for PHP Program Enrollment Considers:

- UAC ages 12 to 17 who would benefit and be responsive to comprehensive short-term therapeutic interventions.
- UAC preparing to step down from acute hospitalization back to a non-restrictive level of care.

PHP Purpose:

- Therapeutic intervention options for youth ages 12 to 17 who are not danger to self or others.
- Supports least restrictive placements and decreases need for RTC admission and program transfers to restrictive levels of care.

The Contractor will be required to provide all personnel, equipment, supplies, facilities, transportation, tools, materials, supervision, and other items and non-personal services necessary to provide for each of

the special population levels of care as defined in ORR UAC Bureau Policy and Field Guidance.
<https://acf.gov/orr/policy-guidance/unaccompanied-children-bureau-policy-guide>

4.0 GOAL

The goal of this RFI is to identify availability of state-licensed special population levels of care facilities (as described within this RFI), capable and interested in providing a network of residential beds for UAC who meet the criteria for placement in respective levels of care.

The interested vendor shall include in a Capability Statement:

1. Description of your organization's current and/or past experience (within the last five years) successfully operating state-licensed special population residential care facilities for minors, including description of existing facilities and programming with capacity to provide a minimum number of beds per special population level of care as described in this RFI; and/or description of proposed facility locations, partners, and capacity to operate.

For each facility type, please provide:

- Facility details: Location(s), capacity, population served, and length of time in operation.
 - If all special population levels of care would not be available on a same campus; include a description of locations for each level of care and how the campus locations are associated; description of cohesive geographic location; and how locations can coordinate services.
- Licensing and accreditation status: Current state licensing and national accreditation if accredited and specify accrediting body, e.g., Joint Commission, COA, etc.
- Performance outcomes: key metrics demonstrating successful operation (e.g., safety records, program completion rates, compliance history).
- Timeline with which respective beds can be available for new admissions upon an award; in cases of longer than 120 calendar days of contract award provide detailed explanation of the challenges and roadblocks.
- Compliance history: A complete statement disclosing any past or current issues with state licensing or national accreditation within the last five years, including:
 - Nature and date of the issue(s)
 - Current status (resolved, pending, ongoing)
 - Corrective actions taken and resolution outcomes
 - Any conditions, restrictions, or monitoring requirements currently in place.

2. Describe your organization's current and/or recent past experience (within the last five years) successfully operating federal programs and contracts.
3. The Government anticipates a Firm-fixed Unit Price per bed contract structure where only 'occupied' beds are invoiced and paid. Provide a detailed breakdown of the level of care per diem (fixed daily bed rate) funding policy. Specifically, information on payment structure for occupied beds and methodology for ensuring the ability to provide the required capacity of minimum number of each special population level of care beds e.g., associated daily rates (e.g., firm-fixed unit price rates) and what is included in the daily bed rate; as well as methodology for funding unoccupied or "held" beds.
4. Provide: (a) Company name; (b) address; (c) Unique Entity Identification (UEI) number; (d) company point(s) of contact, phone number(s), and email address(es); and (e) business type (e.g., small business/other than small business, 8(a), small-business woman-owned, service-disabled veteran-owned small business, etc.)

Programming will:

1. Provide state-licensed care facilities which meet the following ORR ratios for staffing, and are humane and secure environments for UAC who meet criteria for a respective level of care.
 - One (1) on-duty staff for every four (4) children during waking hours;
 - One (1) on-duty staff for every twelve (12) children during sleeping hours;
 - One (1) case manager for every six (6) children;
 - One (1) clinician for every six (6) children; and
 - Two (2) staff present for each one (1) child during secure transport outside the facility; at least one (1) staff present must be of the same gender as the child being transported.
2. Maintain capacity for timely admission of UAC within 72 hours of notification from ORR.
3. Ensure provision of comprehensive services, required under 45 CFR §§ 410.1302, 410.1306, 410.1307, 410.1310, 410.1311, and the UAC Policy Guide.
4. Employ policies and service plans to avoid rejection or ejection of a UAC who meets criteria and is referred for placement.
5. Provide comprehensive services including UAC assessment, diagnosis, treatment, education, nutrition, recreation, religious access, acculturation services, discharge planning, transfer recommendations to less-restrictive settings, and safe release recommendations to an ORR-vetted and approved sponsor, required under relevant sections of [45 CFR Part 410](#) and the UAC Policy Guide.
6. Facility and operations must be compliant with all applicable federal, state, and local regulations for provision of residential secure care for juveniles.
7. Facility (indoor and outdoor) and operations must have capacity to serve the minimum number of UAC for each level of care and must operate 24/7, 365 days per year (366 days during leap year).

5.0 LOCATION OF SERVICE

Special population continuum of care facilities and programming will be performed within the contiguous United States, at the contractor location(s), including any contractor site designated to provide physical custody and direct care. All levels of care within the continuum shall be provided within a cohesive geographical location to ensure continuity of care and ease of transfer between levels of care, as necessary.

These facilities are required to be within a geographic area that can provide access to specialized services that children placed in these settings may need. This is particularly important to ensure children with identified disabilities are able to receive the services, support, and accommodations needed to support their placement in the least restrictive, most integrated environment in their best interest.

6.0 ANTICIPATED PERIOD OF PERFORMANCE

One 12-month base period, with four 12-month optional periods based on the needs of the Government.

7.0 IMPORTANT CONSIDERATIONS

The Government is interested in potential vendors:

1. With experience in providing the full continuum of special population levels of care, as described in this RFI and experience with network management and applying network management business strategies in operating programs for youth, similar to the services described in 45 CFR part 410.
2. With ready or near-ready bed capacities for special population levels of care, as described in this RFI with expected ramp-up not to exceed 120 calendar days from date of an award.
3. Currently operating state-licensed programs with preference for nationally accredited facilities.

Approximately 10 calendar days after responses are due from this Request for Information, ORR intends to hold a live virtual follow-up roundtable discussion with industry to further gather industry's input into ORR's requirement and industry's perspectives. Additional information regarding this event will be posted on this notice.

Notes:

1. Vendors with interest and capability in providing only programming or only the facilities portion of this requirement are encouraged to respond; and encouraged to describe any existing partnerships with state-licensed special population facilities and/or plans to partner.

2. The Government is most interested in vendors with immediate capacity or capable of a timeline not to exceed 120 calendar days to accept new admissions for each of the levels of care as described in this RFI.

Capability Statement Inquiries/Submission Instructions:

1. No phone calls will be accepted. However, written questions may be submitted to Contract Specialist Marvin Barnett at marvin.barnett@hhs.gov and Contracting Officer Sharee Richardson at sharee.richardson@hhs.gov. All questions must be received by 10:00 a.m. Eastern Time on Thursday, April 9, 2026.
2. The Capability Statement shall be limited to 10 double-spaced pages total. The pages will have 1-inch margins, and using not less than 12-point font, Times New Roman.
3. Tables and figures can be Times New Roman and no smaller than 9-point font. The total number of tables and figures shall be limited to 12. The Cover Letter, Tables of Contents, and Technical Appendix are not included in the page count.
4. Capability Statements shall be submitted via email to the Contract Specialist, Marvin Barnett at: marvin.barnett@hhs.gov and Contract Officer, Sharee Richardson at sharee.richardson@hhs.gov. All questions must be received by 10:00 a.m. Eastern Time on Thursday, April 9, 2026.

7.0. DISCLAIMER AND IMPORTANT NOTES

This notice does not obligate the Government to award a contract, and no reimbursement will be made for any costs associated with providing information in response to this RFI. The Government reserves the right to use information provided by the respondents for any purpose deemed necessary and legally appropriate. Any organization responding to this notice should ensure that its response is complete and sufficiently detailed. Information provided will be used to assess tradeoffs and alternatives available for the potential requirement and may lead to the development of solicitation. Respondents are advised that the Government is under no obligation to acknowledge receipt of the information received or provide feedback to respondents with respect to any information submitted. Any solicitation resulting from the analysis of information obtained will be announced to the public in System for Award Management (SAM) in accordance with FAR Part 5. However, responses to this notice will not be considered adequate responses to a solicitation.

Confidentiality

No proprietary, classified, confidential, or sensitive information should be included in your response. The Government reserves the right to use any technical information received in response to this RFI in any resultant solicitation(s).